Blume — Product Documentation

Your feminine intelligence companion. Built for every phase, every body, every season of womanhood.

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The Origin Story

Blume was born from a simple, frustrating observation: **women have been using generic productivity tools, healthcare apps, and wellness trackers designed without them in mind.**

Most period tracking apps stop at logging flow and predicting the next date. Most pregnancy apps are cluttered with ads and offer generic advice. Most health apps treat the female body as a slight variation of the default male body — rather than something with its own profound, rhythmic intelligence.

The name “**Blume”** is a German noun meaning **"flower"** or **"blossom,"** often symbolizing beauty, nature, and growth. Key synonyms include bloom, blossom, flower, and floral.

The question that started this project was:

"What if an app actually understood the female body — not just tracked it?

That one question became three modules, dozens of features, and an entirely different philosophy of how a women's health product should work.

The Problem We're Solving

Women's health has historically been underfunded, understudied, and underserved by technology. Here is what the landscape looks like today:

- **Period tracking apps** (Clue, Flo, Period Tracker) log dates and predict cycles but offer little intelligence around how to *live* around those phases. They are reactive, not proactive.
- **Pregnancy apps** (What to Expect, Ovia) provide information week by week but operate as passive encyclopedias — they don't adapt to the individual, flag anomalies, or build an integrated care picture.
- **Doctor finding platforms** (Practo, Google Maps) are not personalised to symptom patterns. A woman experiencing PCOS symptoms has no guided path to the right specialist — she has to know what she's looking for before she can search for it.

The result is a fragmented experience. A woman might use four or five separate apps for things that are fundamentally connected: her cycle, her mood, her symptoms, her pregnancy, her doctors.

- **Blume brings all of this under one roof — and makes it intelligent.**

The Vision

Blume is not a tracker. It is a **feminine intelligence layer** — a companion that reads the patterns of a woman's body and translates them into actionable guidance for her health, her work, and her life.

The long-term vision has three layers:

- **Layer 1 — Know your body.** Understand your cycle, symptoms, and hormonal patterns with more depth than any current app offers.
- **Layer 2 — Optimise your life.** Use that biological intelligence to plan your week, schedule your tasks, and show up as your most effective self — because your hormonal phases are not obstacles; they are assets.
- **Layer 3 — Connect to care.** When something feels wrong, get guided to exactly the right specialist, with the right questions ready, before you even walk in the door.

How the Idea Came Together

The concept emerged from the intersection of three major trends happening simultaneously in health technology:

- **Cycle syncing gaining mainstream traction.** Books like *In the Flo* by Alisa Vitti introduced the idea of aligning life, work, and diet with the menstrual cycle. The concept had scientific grounding in how estrogen and progesterone fluctuate across the four phases and affect cognition, energy, and social behaviour. But there was no app that implemented this practically at a daily planning level.
- **AI becoming genuinely useful in healthcare.** Large language models reached a point where they could synthesise symptom patterns, compare them against clinical literature (Rotterdam Criteria for PCOS, DSM-adjacent criteria for PMDD), and offer evidence-based guidance — not a diagnosis, but a meaningful, personalised signal. This made a symptom-based disease predictor not just possible but responsible.
- **India-specific care gap.** In India, finding a specialist gynaecologist for a specific condition (fertility, PCOS, endometriosis, menopause) is genuinely difficult. Practo exists but is not symptom-aware. Google Maps is not filtered by specialty. There was a clear, underserved need for a guided, symptom-to-

specialist pipeline — especially one that could operate in Indian cities with their specific geography, languages, and insurance context.

These three threads — cycle syncing, AI symptom intelligence, and care access — became the three modules of Blume.

Product Philosophy

Blume is built on five core beliefs:

- **1. Biology is not a limitation — it is a superpower.**

The menstrual cycle is not something to manage or hide. It is a source of rhythmic intelligence. Blume treats it as such.

- **2. Personalisation over population averages.**

The average cycle is 28 days. Most women's cycles are not 28 days. Blume learns the individual's patterns and predicts based on their history — not a statistical average.

- **3. Proactive, not reactive.**

Rather than waiting for a symptom to become a crisis, Blume surfaces patterns early — in cycle irregularity, mood shifts, or symptom clusters — and nudges toward care before urgency arrives.

- **4. Evidence-based, not fearmongering.**

All AI-generated health insights are grounded in peer-reviewed sources, clinical criteria, and validated screening tools. Blume is always clear: it provides signals, not diagnoses.

- **5. Beautiful enough that women actually use it.**

A health tool women are embarrassed to open is a tool that fails them. Blume is designed to feel like a sanctuary — soft, calm, feminine, and luxurious to interact with.

User Personas

Priya, 26 — The Cycle-Aware Professional

Works at a design agency in Bangalore. Irregular cycles, occasional cramps. Starting to read about cycle syncing and wants a practical way to implement it at

work. Has noticed she is sharper creatively some weeks and completely drained others — but doesn't know why or when to expect it.

- **Blume helps her:** Understand which phase she's in, plan her deep work during follicular phase, schedule client presentations during ovulatory phase, and batch admin tasks in luteal.

Ananya, 31 — Trying to Conceive

Married, living in Mumbai. Has been trying to conceive for eight months. Notices some symptoms she's read are associated with PCOS but hasn't had a formal diagnosis. Doesn't know which type of specialist to see or what questions to ask.

- **Blume helps her:** Run a symptom-based health predictor, identify a likely PCOS pattern, get directed to a reproductive endocrinologist nearby, and walk in with the right questions prepared.

Meera, 29 — First Trimester, First Pregnancy

Eight weeks pregnant. Overwhelmed by the amount of information online. Can't tell which symptoms are normal and which need attention. Partner wants to be involved but doesn't know how.

- **Blume helps her:** Follow a week-by-week development log, ask the AI companion whether her symptoms are normal, track her appointments, and share relevant weekly updates with her partner.

Nisha, 42 — Perimenopause

Notices irregular cycles, hot flushes, sleep disruption. Has been dismissed by a general practitioner. Wants to find a menopause specialist but doesn't know how to filter for one.

- **Blume helps her:** Log and track her changing cycle patterns, identify that her symptom cluster aligns with perimenopause, and find a menopause specialist gynaecologist in her city.

Architecture & Implementation Overview

Blume is structured as a three-module product that shares a common data layer — the user's health profile.

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User Profile (central data store)
│
├── Cycle data (period logs, symptom logs, mood logs)
├── Pregnancy data (week, symptoms, appointments)
├── Doctor favourites & appointment history
└── Preferences (sharing settings, alerts, birth plan)
```

Each module reads from and writes to this central profile. This is what makes the product coherent — your symptom history informs your disease predictor, which informs your specialist recommendation, which connects to your appointment tracker.

- **AI layer.** Blume uses a large language model for three purposes: the weekly plan generator (prompt engineering against cycle phase context), the disease predictor (symptom pattern matching against clinical criteria with confidence scoring), and the pregnancy Q&A (evidence-based question answering with source attribution).
- **External integrations.** Google Places API powers the specialist search and clinic card data. Practo deeplinks handle appointment booking (India-specific). The AI API (Claude / GPT-4 class) handles all generative features.
- **Data privacy architecture.** All health data is encrypted at rest and in transit. Partner sharing uses a permission layer — the user explicitly selects what data to share and can revoke at any time. No health data is used to train models without explicit opt-in consent.

Module 1 — Cycle Intelligence & Life Optimisation

Overview

Module 1 is the core of Blume. It transforms period tracking from a passive log into an active life-optimisation tool. It understands where a woman is in her cycle and uses that understanding to guide her work, energy, and health decisions every single day.

Core Features

- **Period Tracker & Predictors**

Users log the start and end of their period, along with daily symptoms. Blume uses this historical data to predict future cycle dates and phases. Predictions improve over time as more cycles are logged. Accuracy is shown transparently (e.g., ± 2 days).

- **Phase-Aware Calendar**

The calendar displays every day with a colour-coded phase band — menstrual (rose), follicular (warm yellow), ovulatory (sage green), luteal (lavender). Users see their entire month at a glance with biological context built in. The four phases each carry different hormonal profiles:

- Menstrual (Days 1–5): Low estrogen and progesterone. Rest, reflection, admin.
- Follicular (Days 6–13): Rising estrogen. Sharp focus, creativity, learning.
- Ovulatory (Days 14–16): Estrogen peak, LH surge. Communication, leadership, social.
- Luteal (Days 17–28): Progesterone dominant. Detail work, editing, completion.

- **Task Tagging with Phase-Fit Warnings**

Tasks are tagged by type: deep work, social, creative, or admin. The app surfaces a warning when a task type conflicts with the current phase — for example, scheduling a deep-focus writing project during the menstrual phase, when cognitive energy is typically lower. Warnings are gentle suggestions, not restrictions.

- **Daily Briefing Card**

Every morning, the app opens to a daily briefing: "Day 11, Follicular phase. Peak focus window today. Estrogen is rising — ideal for learning, starting new projects, and strategic thinking." The briefing pulls from phase science, the user's logged mood patterns, and any logged symptoms to make it feel personal, not generic.

- **Quick Energy & Mood Logging**

A lightweight pop-up system — triggered daily or on demand — asks the user to select a mood (energised, calm, tired, irritable, emotional, crampy) and log their energy level on a 1–10 slider. This takes under ten seconds. Over time it builds a

longitudinal picture of how mood and energy track against cycle phases, revealing the user's individual patterns.

- **AI Weekly Plan Generator**

The user pastes their task list for the week. The AI reads the current phase, the upcoming phase transitions, and the task types, then returns a phase-optimised schedule. Deep work tasks are pushed to follicular days. High-stakes meetings and presentations are placed during ovulatory windows. Editing and administrative work fills the luteal window. The generator explains its reasoning so the user builds intuition over time.

- **PCOS, PMS & Disease Predictor**

The user selects symptoms from a comprehensive list (irregular periods, acne, excess hair growth, weight gain, pelvic pain, severe cramping, absence of period, mood extremes, and more). The AI compares this symptom cluster against clinical criteria for PCOS (Rotterdam Criteria), PMDD (DSM-5 criteria), endometriosis markers, thyroid-related patterns, and others. It returns a likelihood score for each condition with a brief explanation grounded in evidence. It then recommends the appropriate specialist type and links directly to Module 3 to find one.

Enhanced Features

- **Partner & Family Sharing**

Users can share specific, curated cycle information with a partner or family member — fertile window, phase, general mood indicator — via a permission-gated sharing panel. Sensitive symptom details are never shared without explicit selection. This helps partners understand cycle-related mood shifts without requiring the user to explain every time.

- **Detailed Pain Mapping**

Beyond logging that pain exists, users can specify type (cramps, back pain, headache, joint pain), intensity on a 1–10 scale, and location on a simple body diagram. This creates a richer dataset for the disease predictor and provides more useful information to share with a gynaecologist.

- **Historical Data Insights**

After three or more cycles are logged, Blume surfaces deeper pattern insights: average phase lengths, consistent energy peaks, recurring symptom timing, mood patterns correlated with cycle days. These are presented as gentle monthly summaries — "You tend to feel most energetic on days 8–12. Your luteal phase averages 13 days, slightly longer than typical."

- **Stress & Sleep Correlation**

Users can optionally log stress levels (low / moderate / high) and sleep hours. Over time, Blume surfaces correlations — "Your cycle was 3 days longer in months where you logged high stress consistently." This connects lifestyle data to cycle health in a meaningful way.

- **Medication Tracker**

Log medications with dosage and frequency. Blume can surface patterns — for example, whether taking a particular supplement correlates with reduced symptom severity. Useful both for personal insight and for conversations with a doctor.

- **Customisable Alerts**

Beyond built-in phase reminders, users can set custom alerts: start prenatal vitamins at cycle day 1, order period supplies three days before predicted start, remind partner of luteal phase beginning. The alert system is flexible and non-intrusive.

Module 2 — Pregnancy Companion

Overview

Module 2 is a complete, week-by-week pregnancy companion. It takes a woman from the moment she calculates her due date through labour preparation and into postpartum planning. It combines developmental information, symptom tracking, nutrition guidance, appointment management, and an AI companion into a single, calm, evidence-based experience.

Core Features

- **Due Date Calculator**

Calculates estimated due date from either the last menstrual period (LMP) or a known conception date. Displays gestational age in weeks and days, trimester,

and percentage of pregnancy complete. Automatically populates the appointment reminder timeline.

- **Weekly Development Log**

Each week, the user sees: baby's approximate size (communicated as a familiar object — blueberry at week 7, avocado at week 16, etc.), key organ and developmental milestones for that week, what physical changes to expect in the mother's body, and what emotional shifts are common. The language is warm and grounding, not clinical.

- **Symptom Tracker by Trimester**

Organised by trimester, users log symptoms: nausea, fatigue, heartburn, swelling, breast tenderness, kick counts (third trimester), and more. The tracker distinguishes between common expected symptoms and those that warrant medical attention — surfacing a gentle flag when a symptom pattern falls outside normal ranges.

- **Appointment & Scan Reminder Timeline**

A visual timeline of key appointments across all forty weeks: dating scan at eight weeks, NT scan and NIPT at eleven to thirteen weeks, anomaly scan at twenty weeks, gestational diabetes test at twenty-four to twenty-eight weeks, Group B Strep test at thirty-six weeks, and weekly check-ins from thirty-six weeks onward. Users receive reminders in advance and can mark appointments as completed.

- **Nutrition & Supplement Guide per Trimester**

Each trimester has distinct nutritional needs. The app provides guidance specific to each phase:

- First trimester: Folic acid (400–800mcg daily), Vitamin B6 for nausea, food aversions management.
- Second trimester: Iron supplementation, Vitamin D, calcium, increasing caloric intake.
- Third trimester: Omega-3 for brain development, continued iron, hydration emphasis, foods to support labour preparation.
- **AI Q&A Companion**

The user can type any question — "Is it normal to have no symptoms at week 9?", "Is the cramping I feel in week 14 okay?", "Can I eat sushi during pregnancy?" — and receive a clear, evidence-based answer with appropriate caveats. The AI is calibrated to be reassuring where science supports reassurance, and to direct toward a doctor when a symptom pattern warrants it. Every answer notes that it supplements, not replaces, medical advice.

- **Weight Gain Tracker**

Users log their weight weekly. Blume overlays a healthy weight gain range specific to their pre-pregnancy BMI (underweight, normal, overweight, obese — per ACOG guidelines). The tracker shows their actual line against the recommended range with a simple, readable chart. Designed to be informative without being anxiety-inducing.

- **Birth Plan Builder**

A guided, structured tool for capturing birth preferences across six categories: birth setting (hospital, birthing centre, home), labour preferences (positions, mobility, water), pain relief (epidural, gas and air, natural), support persons, newborn preferences (delayed cord clamping, skin-to-skin, vitamin K), and postpartum preferences (feeding, postpartum support, mental health check-in plan). The completed birth plan can be exported as a PDF to share with a care team.

Enhanced Features

- **Partner Involvement**

A dedicated section — viewable by a shared partner — provides weekly updates framed for the non-pregnant parent: what is happening with the baby this week, what the pregnant partner may be experiencing, and three to five specific, practical things the partner can do this week (cook iron-rich meals, attend the scan, research pram options). This is one of the most underserved areas of pregnancy apps.

- **Exercise Guidance**

Phase-appropriate and trimester-appropriate exercise suggestions: walking, prenatal yoga, swimming, pelvic floor exercises. Adjusts recommendations based

on trimester and any flagged symptoms (symphysis pubis dysfunction, low-lying placenta, etc.).

- **Contraction Timer**

A clean, single-purpose contraction timer that logs frequency (time between contractions) and duration (length of each contraction). Provides guidance on when patterns indicate it may be time to contact a midwife or head to hospital (the 5-1-1 rule).

- **Mental Health Check-ins**

Brief, validated screening questions drawn from the Edinburgh Postnatal Depression Scale (EPDS) and PHQ-2, administered at clinically relevant points — end of first trimester, third trimester, and postpartum. If responses indicate elevated risk, Blume gently surfaces mental health resources and encourages speaking with a care provider. Done with care, not alarm.

- **Local Resources**

Surfaced alongside the specialist finder: lactation consultants, postpartum doulas, antenatal classes, and paediatrician recommendations — filtered by the user's city and pin code.

Module 3 — Care Connection

Overview

Module 3 closes the loop between health intelligence and healthcare access. It takes the symptom patterns and health signals from Modules 1 and 2 and translates them into guided specialist access — helping a woman find the right doctor, show up prepared, and never leave a consultation having forgotten to ask the most important questions.

Core Features

- **Symptom-Based Specialist Routing**

When a user completes the disease predictor in Module 1 or flags a significant pregnancy symptom in Module 2, Blume surfaces a specialist recommendation — not just "see a gynaecologist," but "based on your PCOS symptom pattern, a Reproductive Endocrinologist or PCOS-specialist OB-GYN is most appropriate." This guided routing is what differentiates Blume from a generic doctor directory.

- **Location Search**

Search by city, area name, or PIN code. The search pulls real clinic data via Google Places API, returning results closest to the user's location with accurate walking and driving distances.

- **Specialty Filters**

Filter by: Pregnancy / Obstetrics, PCOS & Hormonal Health, Fertility & Reproductive Endocrinology, Menopause & Perimenopause, General OB-GYN, Adolescent Gynaecology, Endometriosis. Sub-specialty filters allow more precise matching — for example, filtering specifically for laparoscopic endometriosis surgeons or fertility specialists who practice IUI/IVF.

- **Clinic Cards**

Each result displays: doctor name and photo, specialty, clinic name and address, distance from the user, ratings and review count, clinic hours, and walk-in availability. Cards are clean, scannable, and prioritise the information a woman needs to make a quick decision.

- **Direct Booking**

A "Book appointment" button links directly to the doctor's Practo profile (primary, India-specific) with a Google Maps fallback for navigation. Where real-time availability is supported, the integration shows available appointment slots.

- **Save to Favourites**

Users can save doctors to a personal list within their profile. Saved doctors appear in a quick-access panel and can be tagged with notes ("recommended by my sister", "fertility specialist only").

- **AI-Generated Pre-Appointment Questions**

Before a scheduled appointment, Blume generates a tailored list of questions for the user to ask, based on their current cycle phase, symptom log, and the specialist type. Examples:

- For a PCOS consultation: "Could my elevated androgens be causing my hair thinning? Should I test AMH?"

- For a pregnancy scan: "Is the baby's position affecting what you can see on the scan?"
- For a menopause consultation: "At what point would HRT be appropriate for my symptom severity?"

Users can edit, add to, and save this list. They can also print or share it before walking in.

Enhanced Features

- **Teleconsultation Filter**

Filter the specialist list for doctors offering online video consultations — useful for follow-ups, second opinions, or users in smaller cities with limited access to subspecialists.

- **Doctor Profiles**

Expanded profile pages: medical education, years of experience, languages spoken, hospital affiliations, areas of specific expertise, publications or media mentions. This helps users evaluate a specialist before committing to an appointment.

- **Real Patient Reviews**

Curated genuine reviews from verified patients — moderated to remove spam but not sanitised to remove critical feedback. Reviews are displayed with recency filtering and can be tagged by condition (e.g., "reviewed as a PCOS patient").

- **Real-Time Appointment Availability**

Where API integration with Practo or clinic booking systems is possible, show live available slots. Otherwise, display estimated wait times based on historical data.

- **Follow-Up Reminders**

If a user books an appointment or notes a consultation in the app, Blume automatically creates a follow-up reminder prompt — "It's been six weeks since you saw Dr Priya Menon. Was a follow-up recommended?" This prevents the common experience of leaving a consultation with a plan that then gets lost.

- **Referral Network**

After a consultation is logged, Blume surfaces related specialists who might be relevant. For example, after seeing a PCOS specialist, the app might suggest: "Many women with PCOS also work with a dietitian specialising in insulin resistance. Would you like to find one?"

Roadmap

Phase 0 — Foundation (Weeks 1–4)

The focus here is core infrastructure and the MVP of Module 1.

Building the user profile and health data model. Designing and implementing the phase-aware calendar. Building the period logging flow with symptom selection. Implementing the daily briefing card with static phase content. Setting up the AI API integration. Launching the MVP with ten internal testers.

Phase 1 — Module 1 Full Build (Weeks 5–10)

Complete all Module 1 core features. Launch the mood and energy logging system. Implement cycle prediction algorithm (based on user's personal history after two logged cycles). Build the task tagging system with phase-fit warnings. Launch the AI weekly plan generator. Soft launch to a closed beta group of fifty women. Collect feedback intensively.

Phase 2 — Disease Predictor & Module 3 Core (Weeks 11–16)

Build the PCOS/PMS/disease predictor with clinical criteria matching and confidence scoring. Integrate Google Places API for specialist search. Build doctor card UI and filter system. Implement Practo deeplink booking. Launch AI-generated pre-appointment question generator. Expand beta to two hundred users.

Phase 3 — Module 2 Build (Weeks 17–24)

Build the full pregnancy companion. Due date calculator and weekly development log. Symptom tracker by trimester. Appointment timeline. Nutrition guide. Weight gain tracker. AI Q&A companion. Birth plan builder. Begin integration of partner sharing for pregnancy module.

Phase 4 — Enhancement Layer (Weeks 25–36)

Add all enhanced features across modules: detailed pain mapping, historical insights, stress and sleep correlation, medication tracker, partner involvement

features, teleconsultation filter, real patient reviews, contraction timer, mental health check-ins, local resource integration, follow-up reminders.

Phase 5 — Scale & Monetisation (Months 10–12)

Move from closed beta to public launch. Implement subscription model (free tier with core tracking, premium tier with AI features and full care connection). Begin partnerships with hospital groups and insurance providers in India. Explore B2B offering for employee wellness programs.

Future Considerations (Post Year 1)

Wearable integration (Fitbit, Apple Watch, Oura Ring) for passive data collection. Lab result uploads (hormone panels, AMH, thyroid). Menopause module — dedicated tracking and care pathway for perimenopause and menopause. International expansion beyond India, with locale-specific care connection.

Tech Stack Considerations

These are recommendations based on the product architecture. Final decisions should be validated by the engineering team.

- **Frontend:** React Native (cross-platform iOS and Android from a single codebase). Tailwind CSS for consistent design tokens. Reanimated for smooth, fluid animations.
- **Backend:** Node.js with Express or FastAPI (Python) if the AI layer benefits from Python-native libraries. PostgreSQL for relational health data. Redis for session and caching. HIPAA/DPDP-compliant data handling architecture.
- **AI Layer:** Claude (Anthropic) or GPT-4 class model via API for the generative features. Prompt templates version-controlled and tested. Response guardrails to prevent the AI from making diagnostic claims.
- **External APIs:** Google Places API (specialist search). Practo API or deeplink (appointment booking). Push notification service (Firebase Cloud Messaging).
- **Authentication:** Phone number OTP (India-first UX). Optional biometric unlock for sensitive health data access.
- **Storage:** Encrypted at rest (AES-256). Health data segmented from PII in database architecture. No health data logged in third-party analytics.

Design Principles

Blume’s visual and interaction design is guided by five principles:

- **Calm over clinical.** Healthcare apps often feel like hospital waiting rooms — sterile, anxious, transactional. Blume feels like a sanctuary. Soft rose, lavender, sage, and peach. Generous whitespace. Typography that breathes.
- **Progressive disclosure.** The most important information is always surface-level. Detail is one tap deeper. A woman in a hurry sees her phase, her briefing, her day. A woman who wants to go deeper can explore her symptom history, her predictor results, her doctor profiles.
- **Earned trust.** Every AI-generated insight is labelled clearly as AI-generated. Every health claim is sourced. The app never overpromises. When it says "this may indicate PCOS," it explains what that means, what the confidence is based on, and what the next step should be.
- **Designed for women, by women.** The language, the imagery, the assumptions built into the UX — all of these should reflect the actual diversity of women's experiences. Not one body type, not one life stage, not one cultural context.
- **Accessibility as standard.** Screen reader support, high contrast mode, font size adjustment — these are not optional. Women using Blume may be managing chronic conditions, recovering from surgery, or navigating pregnancy brain. The app should work for all of them.

Open Questions & Future Thinking

These are the questions the team should hold as the product develops. They don't have clean answers yet — and that's intentional.

- **On the disease predictor:** At what point does a symptom-matching AI tool become a liability if users over-rely on it? How do we design the confidence scores and disclaimers to be meaningful without creating either complacency or panic?
- **On data sensitivity:** Menstrual data is among the most sensitive health data a person can share, particularly in contexts where access to reproductive healthcare is legally contested. How do we architect Blume's data storage and

sharing policies to protect users in the most adversarial possible regulatory environments?

- **On partner sharing:** The cycle-sharing feature assumes a supportive partner context. What are the risks for users in controlling or coercive relationships? Should there be a "safe exit" feature that obscures sharing history?
- **On the AI companion in pregnancy:** What happens when a user asks the AI a question that falls outside its competence — a symptom that could indicate something serious? How do we calibrate the AI to escalate appropriately without triggering unnecessary alarm in every interaction?
- **On the India-specific care layer:** Not all cities have the same density of specialists. How does Blume degrade gracefully when Google Places returns no PCOS specialists within fifty kilometres? Does it surface teleconsultation options? Does it surface general OB-GYNs with a note about what to ask?
- **On monetisation ethics:** Blume will need a sustainable business model. Subscriptions are the cleanest model. But what does free-tier access look like, and are we comfortable that women who cannot afford a premium tier still get meaningful value — not a deliberately crippled experience designed to convert them?

*"The menstrual cycle is not a problem to be managed. It is an intelligence to be listened to."**